

§ 170.315(a)(5)

Patient demographics and observations

Certification Companion Guide v1.3

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This Certification Companion Guide (CCG) is an informative document designed to assist with health IT product certification. The CCG is not a substitute for the requirements outlined in regulation and related ONC final rules. It extracts key portions of ONC final rules' preambles and includes subsequent clarifying interpretations. To access the full context of regulatory intent please consult the [Certification Regulations](#) page for links to all ONC final rules or consult other regulatory references as noted. The CCG is for public use and should not be sold or redistributed.

Certification Clarifications

Technical Explanations and Clarifications

- The demographic data can come from other sources (e.g., a registration system or practice management system) so long as testing demonstrates the Health IT Module can perform all required functions included in the Patient demographics and observations certification criterion. [see also [Health IT Certification Program Overview](#)]
- Testing for preferred language using the standard at § 170.207(g)(2) Request for Comments (RFC) 5646 will focus on all the languages present in [ISO 639-2](#).
- Consistent with the § 170.207(g)(2) RFC 5646, the shortest alpha code for the language should be used. Testing will only test the primary language tag and not test for extension components specified in § 170.207(g)(2) RFC 5646 such as extended language sub-tags, script tag, or region tag. [see also [80 FR 16817](#)] Specifically:
 - use alpha 2 character code if one exists (ISO 639-1);
 - use alpha 3 character code if an alpha 2 character code does not exist (ISO 639-2); and

- region extensions (ISO 3166-1) are permitted but not required (however, if a region extension is used, it will be verified for accuracy as part of testing and must be correct).
- ONC's adoption of the "Sex Parameter for Clinical Use", "Name to Use", and "Pronouns" data elements does not establish a requirement for health care providers or patients to record or disclose this information, or use these capabilities. ONC encourages developers of certified health IT to work with providers to develop appropriate workflows. [see also [89 FR 1296](#)]
- Certified Health IT Developers must assess user-facing functionality gaps between Health IT Modules currently certified to the § 170.315(a)(5) criterion and the updated § 170.315(a)(5) criterion requirements as specified in the HTI-1 Final Rule. Certified Health IT Developers must assess if updates to the Health IT Module's user-facing functionalities to resolve these gaps necessitate user-centered design processes applied during development and thus must be included as part of summative testing in accordance with the § 170.315(g)(3) criterion. Accordingly and as necessary, Certified Health IT Developers must update their safety-enhanced design (SED) testing for updated § 170.315(a)(5) criterion requirements.
- Consistent with Executive Order (EO) 14168 and OPM guidance, Health IT Modules certifying and/or currently certified to the demographics certification criteria at 45 CFR 170.315(a)(5) are only required to demonstrate, for conformance with paragraph (a)(5)(i)(C) ("sex"), that it can record sex in accordance with the following SNOMED CT® codes found in the standard specified in § 170.207(n)(2):
 - 248152002 |Female (finding)|; and
 - 248153007 |Male (finding)|.
- Health IT Modules certified to § 170.315(a)(5), are not required to demonstrate conformance with paragraphs:
 - (a)(5)(i)(D) ("sexual orientation");
 - (a)(5)(i)(E) ("gender identity");
 - (a)(5)(i)(F) ("sex parameters for clinical use");
 - (a)(5)(i)(G) ("name to use"); and
 - (a)(5)(i)(H) ("pronouns").
- There is no standard required for recording date of birth (i.e., any format may be used).

Technical outcome –

- A user can record each one of a patient's race(s) and each one of a patient's ethnicity(ies) according to concepts in the CDC Race and Ethnicity (CDCREC) code system Version 1.2.

- The software must be able to aggregate each one of a patient's race(s) and each one of a patient's ethnicity(ies) and record the race(s) and ethnicity(ies) according to the § 170.207(f)(1) The Office of Management and Budget (OMB) Standards for Maintaining, Collecting, and Presenting Federal Data on Race and Ethnicity. The categories to which race and ethnicity selections must be aggregated and recorded include:
 - American Indian or Alaska Native;
 - Asian;
 - Black or African American;
 - Native Hawaiian or Other Pacific Islander
 - White;
 - Hispanic or Latino; and
 - Not Hispanic or Latino
 - The user must be able to record whether the patient declined to specify a race, an ethnicity, and both.
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- Note that while this provision focuses on the capture of race and ethnicity, industry standards require race and ethnicity be exchanged as separate fields (e.g., Consolidated Clinical Document Architecture (C-CDA) and Quality Reporting Document Architecture (QRDA)). ONC suggests developers bear this in mind when developing their health IT systems.
 - The CDCREC code system includes over 900 concepts for race and ethnicity. [see also [80 FR 16816](#)] A health IT developer is free to determine how the user interface is designed, including how many race and ethnicity values are displayed. No default minimum number of visible selections is expected or implied. During testing, however, any of the concepts for race and ethnicity may be tested. [see also [80 FR 62618](#)]
 - Health IT must be able to record multiple races or ethnicities reported by a patient and be able to be correctly roll them up to the OMB categories. While the criterion is not prescriptive in that it does not set a “maximum-minimum” number of multiple entries that must be supported, the criterion supports surveillance that could include detailed race data that would cover up to five OMB race categories and two ethnicity categories at once to show multi-race, multi-ethnicity selection, support across the 900+ value set, and ultimately accurate roll-up.
 - The Health IT Module must be able to record multiple races and/or ethnicities for a patient. [see also [80 FR 62618](#)]
 - ONC provides the following object identifier (OID) to assist developers in the proper identification and exchange of health information coded to certain vocabulary standards.
 - CDCREC OID: 2.16.840.1.113883.6.238 [see also [80 FR 62612](#)]
 - Health IT Modules can present for certification to a more recent version of the CDCREC code system than what is presented in regulation. [see also [80 FR 62612](#)]
 - The concepts in the CDCREC code system are mapped to the race and ethnicity categories in the § 170.207(f)(1) The Office of Management and Budget (OMB) Standards for Maintaining, Collecting, and Presenting Federal Data on Race and Ethnicity. Testing will verify that the granular race and ethnicity

values are correctly mapped to the OMB categories. The mapping of these codes uses the hierarchical relationships defined in [CDCREC](#).

- The health IT must be able to represent race and ethnicity categories in the § 170.207(f)(1) The OMB Standards for Maintaining, Collecting, and Presenting Federal Data on Race and Ethnicity standard using CDCREC. The use of more granular values is permitted, so long as they map to the OME categories.
- The § 170.207(f)(1) The OMB Standards for Maintaining, Collecting, and Presenting Federal Data on Race and Ethnicity standard provides two formats for the collection of race and ethnicity data. The first format collects data using two separate questions. The second format, the “combined format”, collects data using a single question. Implementation using either format is acceptable for the purposes of certification.
- A product does not need to display all the race and ethnicity values to meet the certification criterion. The health IT developer has the discretion to create a default selection set or enable customization choices for providers. However, for the purposes of testing and surveillance, a developer should be prepared to show that the product can represent any of the races and ethnicities in the value set created by the standard.

Technical outcome – A user can record a patient’s preferred language according to the § 170.207(g)(2) RFC 5646 standard. The user must be able to record whether the patient declined to specify preferred language.

- § 170.207(g)(2) RFC 5646 is compatible with C-CDA Release 2.1 and ISO 639-1, 639-2, and 639-3 can be mapped to it. [see also [80 FR 62619](#)]
- ONC provides the following OID to assist developers in the proper identification and exchange of health information coded to certain vocabulary standards.
 - Tags for Identifying Languages – § 170.207(g)(2) RFC 5646 code system OID: 2.16.840.1.113883.6.316 [see also [80 FR 62612](#)]
- A product does not need to display all of the language codes to meet the certification criterion. The health IT developer has the discretion to create a default selection set or enable customization choices for providers. However, for the purposes of testing, a developer should be prepared to show that the product can record any of languages in the value set created by the standard.

Technical outcome – A user can record a patient’s sex according to the SNOMED CT® U.S. Edition codes specified at § 170.207(a)(1).

- No additional clarifications

Technical outcome – A user can record a patient’s sexual orientation and gender identity according to the SNOMED CT® U.S. Edition codes specified at § 170.207(a)(1). The user must be able to record whether the patient declined to specify sexual orientation and/or gender identity.

- Consistent with EO 14168, Health IT Modules certified to § 170.315(a)(5) are not required to demonstrate conformance with this paragraph.

Technical outcome – A user can record a patient’s sex parameter for clinical use according to the LOINC® codes specified in the “standard(s) referenced” column.

- Consistent with EO 14168, Health IT Modules certified to § 170.315(a)(5) are not required to demonstrate conformance with this paragraph.

Technical outcome – A user can record at least one preferred name to use.

- Consistent with EO 14168, Health IT Modules certified to § 170.315(a)(5) are not required to demonstrate conformance with this paragraph.

Technical outcome – A user can record at least one pronoun according to the LOINC® codes specified in the “standard(s) referenced” column.

- Consistent with EO 14168, Health IT Modules certified to § 170.315(a)(5) are not required to demonstrate conformance with this paragraph.

Technical outcome – For inpatient settings only, a user must be able to record, change, and access a patient’s preliminary cause of death and date of death.

- No standard is required for recording preliminary cause of death and free text is permitted for certification. [see also [77 FR 54209](#) and [80 FR 62619](#)]

Regulation Text

§ 170.315 (a)(5) Patient demographics and observations—

(i) Enable a user to record, change, and access patient demographic and observations data including race, ethnicity, preferred language, sex, sex parameter for clinical use, sexual orientation, gender identity, name to use, pronouns and date of birth.

(A)

- (1) Enable each one of a patient's races to be recorded in accordance with, at a minimum, the standard specified in § 170.207(f)(3) and whether a patient declines to specify race.
- (2) Enable each one of a patient's ethnicities to be recorded in accordance with, at a minimum, the standard specified in § 170.207(f)(3) and whether a patient declines to specify ethnicity.
- (3) Aggregate each one of the patient's races and ethnicities recorded in accordance with paragraphs (a)(5)(i)(A)(1) and (2) of this section to the categories in the standard specified in § 170.207(f)(1).

(B) Enable preferred language to be recorded in accordance with the standard specified in § 170.207(g)(2) and whether a patient declines to specify a preferred language.

(C) Enable sex to be recorded in accordance with the standard specified in § 170.207(n)(1) for the period up to and including December 31, 2025; or § 170.207(n)(2).

- (D) Enable sexual orientation to be recorded in accordance with, at a minimum, the version of the standard specified in § 170.207(o)(1) for the period up to and including December 31, 2025; or § 170.207(o)(3), as well as whether a patient declines to specify sexual orientation.
- (E) Enable gender identity to be recorded in accordance with, at a minimum, the version of the standard specified in § 170.207(o)(2) for the period up to and including December 31, 2025; or § 170.207(o)(3), as well as whether a patient declines to specify gender identity.
- (F) Enable at least one sex parameter for clinical use to be recorded in accordance with, at a minimum, the version of the standard specified in § 170.207(n)(3). Conformance with this paragraph is required by January 1, 2026.
- (G) . Enable at least one preferred name to use to be recorded. Conformance with this paragraph is required by January 1, 2026.
- (H) . Enable at least one pronoun to be recorded in accordance with, at a minimum, the version of the standard specified in § 170.207(o)(4). Conformance with this paragraph is required by January 1, 2026.

(ii) Inpatient setting only.

Enable a user to record, change, and access the preliminary cause of death and date of death in the event of mortality.

Standards & References

The following standards are referenced within the certification criteria for: **§ 170.315(a)(5) Patient demographics and observations**

Paragraph (a)(5)(i)(A)

§ 170.207(f)(1) [The Office of Management and Budget Standards for Maintaining, Collecting, and Presenting Federal Data on Race and Ethnicity, Statistical Policy Directive No. 15, as revised, October 30, 1997](#)

§ 170.207(f)(3) [CDC Race and Ethnicity Code Set Version 1.2 \(July 8, 2021\)](#)

Paragraph (a)(5)(i)(B)

§ 170.207(g)(2) [Request for Comments \(RFC\) 5646](#)

Paragraph (a)(5)(i)(C)

Consistent with Executive Order (EO) 14168, Health IT Modules are only required to demonstrate support for recording sex in accordance with the following SNOMED CT® codes found in the standard specified in § 170.207(n)(2):

- 248152002 |Female (finding)|; and
- 248153007 |Male (finding)|.

§ 170.207(n)(2) Sex must be coded in accordance with, at a minimum, the version of SNOMED CT® U.S. Edition codes specified in § 170.207(a)(1)

Paragraphs (a)(5)(i)(D) and (E)

§ 170.207(o)(3) Sexual Orientation and Gender Identity must be coded in accordance with, at a minimum, the version of SNOMED CT® codes specified in § 170.207(a)(1)

Paragraph (a)(5)(i)(F)

Consistent with EO 14168, conformance with this paragraph is no longer required.

§ 170.207(n)(3) Sex Parameter for Clinical Use must be coded in accordance with, at a minimum, the version of LOINC® codes specified in § 170.207(c)(1)

Paragraph (a)(5)(i)(H)

Consistent with EO 14168, conformance with this paragraph is no longer required.

§ 170.207(o)(4) Pronouns must be coded in accordance with, at a minimum, the version of LOINC® codes specified in 170.207(c)(1).

Certification Dependencies

Privacy and Security

This certification criterion was adopted at § 170.315(a)(5). As a result, an ONC Authorized Certification Body (ONC-ACB) must ensure that a product presented for certification to a § 170.315(a) criterion includes the privacy and security criteria (adopted in § 170.315(d)) within the overall scope of the certificate issued to the product.

- The privacy and security criteria (adopted in § 170.315(d)) do not need to be explicitly tested with this specific paragraph (a) criterion unless it is the only criterion for which certification is requested.
- As a general rule, a product presented for certification only needs to be presented once to each applicable privacy and security criterion (adopted in § 170.315(d)) so long as the health IT developer attests that such privacy and security capabilities apply to the full scope of capabilities included in the requested certification. However, exceptions exist for § 170.315(e)(1) “View, download, and transmit to 3rd party (VDT)” and (e)(2) “Secure messaging,” which are explicitly stated.
- § 170.315(d)(2)(i)(C) is not required if the scope of the Health IT Module does not have end-user device encryption features.

For more information on the approaches to meet these Privacy and Security requirements, please download the Privacy and Security CCG below:

Approaches:

If choosing Approach 1 • Authentication, access control, and authorization (§ 170.315(d)(1)). • Auditable events and tamper-resistance (§ 170.315(d)(2)). • Audit reports (§ 170.315(d)(3)). • Amendments (§ 170.315(d)(4)). • Automatic access time-out (§ 170.315(d)(5)). • Emergency access (§ 170.315(d)(6)). • End-user device encryption (§ 170.315(d)(7)). • Encrypt authentication credentials (§ 170.315(d)(12)). • Multi-factor authentication (MFA) (§ 170.315(d)(13)).	If choosing Approach 2 For each applicable privacy and security certification criterion not certified for Approach 1, the health IT developer may certify using system documentation that is sufficiently detailed to enable integration such that the Health IT Module has implemented service interfaces for each applicable privacy and security certification criterion that enable the Health IT Module to access external services necessary to meet the requirements of the privacy and security certification criterion. Please see the 21st Century Cures Act: <i>Interoperability, Information Blocking, and the ONC Health IT Certification Program</i> Final Rule at 85 FR 25710 for additional clarification.
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Design and Performance

The following design and performance certification criteria (adopted in § 170.315(g)) must also be certified in order for the product to be certified.

- [Safety-enhanced design \(§ 170.315\(g\)\(3\)\)](#) must be explicitly demonstrated for this criterion.
- [Quality management system \(§ 170.315\(g\)\(4\)\)](#): When a single quality management system (QMS) is used, the QMS only needs to be identified once. Otherwise, the QMS’ need to be identified for every capability to which it was applied.
- [Accessibility-centered design \(§ 170.315\(g\)\(5\)\)](#): When a single accessibility-centered design standard is used, the standard only needs to be identified once. Otherwise, the accessibility-centered design standards need to be identified for every capability to which they were applied; or, alternatively, the developer must state that no accessibility-centered design was used.

Certification Companion Guide Changelog

The following changelog applies to:

§ 170.315(a)(5)Patient demographics and observations

Revision History

Version #
1.0
Description of Change
Initial publication
Version Date
03-11-2024

Version #

1.1

Description of Change

- For the entire criterion, added clarification regarding updates to safety-enhanced design testing to comply with HTI-1 Final Rule requirements.
- For Paragraph (a)(5)(i)(F), added clarification regarding requirements for coding Sex Parameter for Clinical Use.
- For Paragraphs (a)(5)(i)(D) and (E), removed a clarification and revised a clarification regarding use of expiring standards.

Version Date

08-12-2024

Version #

1.2

Description of Change

- For entire criterion, added clarification regarding compliance with EO 14168 and OPM guidance.
- For Paragraph (a)(5)(i)(A): Updated how clarifications refer to referenced standards and their content for consistency and accuracy. Updated clarification to mention the “Not Hispanic or Latino” category must be aggregated and recorded according to § 170.207(f)(1). Updated clarification for how to access CDCREC. Revised for further clarity the clarification regarding the “combined format” available in the standard adopted at § 170.207(f)(1).

Version Date

03-27-2025

Version #

1.3

Description of Change

Removal of requirements that expired on January 1, 2026.

Version Date

05-14-2026

